

INSERT MEFUROSAN

(12 APRIL '23) - Exp. Malaysia

**Controlled
Medicine**

MEFUROSAN[®]

**Cream 0.1 % w/w
Mometasone Furoate**

Each gram contains:
Mometasone Furoate 1 mg

PRODUCT DESCRIPTION
White cream, odorless

PHARMACOLOGY

Pharmacology
Mometasone furoate exhibits marked anti-inflammatory activity and marked anti-psoriatic activity in standard animal predictive models. In the croton oil assay in mice, mometasone was equipotent to betamethasone valerate after single application and about 8 times as potent after five applications. In guinea pigs, mometasone was approximately twice as potent as betamethasone valerate in reducing M. ovalis-induced epidermal acanthosis (i.e. anti-psoriatic activity) after 14 applications.

Pharmacokinetics
Pharmacokinetic studies have indicated that systemic absorption following topical application of mometasone furoate cream 0.1% is minimal, the majority of which is excreted within 72 hours following application. Characterisation of metabolites was not feasible owing to the small amounts present in plasma and excreta.

INDICATIONS
MEFUROSAN[®] cream is indicated for the relief of the inflammatory and pruritic manifestations of corticosteroid responsive dermatoses.

RECOMMENDED DOSE
A thin film of **MEFUROSAN[®]** Cream should be applied to the affected skin areas once daily until the lesion heals or for a duration of three weeks, whichever is sooner.

ROUTE OF ADMINISTRATION:
Topical

CONTRA-INDICATIONS
Mefurosan is contraindicated in facial rosacea, acne vulgaris, skin atrophy, perioral dermatitis, perianal and genital pruritis, napkin eruptions, bacterial (e.g. impetigo, pyoderma), viral (e.g. herpes simplex, herpes zoster and chickenpox

verrucae vulgares, condylomata acuminata, molluscum contagiosum), parasitical and fungal (e.g. candida or dermatophyte) infections, varicella, tuberculosis, syphilis or post-vaccine reactions. Mefurosan should not be used on wounds or on skin which is ulcerated. Mefurosan should not be used in patients who are sensitive to mometasone furoate or to other corticosteroids or to any of the excipients.

WARNING AND PRECAUTIONS
If irritation or sensitisation develops with the use of Mefurosan, treatment should be withdrawn and appropriate therapy instituted. Should an infection develop, use of an appropriate antifungal or antibacterial agent should be instituted. If a favourable response does not occur promptly, the corticosteroid should be discontinued until the infection is adequately controlled. Systemic absorption of topical corticosteroid can produce reversible hypothalamic pituitary adrenal (HPA) axis suppression with the potential for glucocorticosteroid insufficiency after withdrawal of treatment. Manifestations of Cushing's syndrome, hyperglycemia, and glucosuria can also be produced in some patients by systemic absorption of topical corticosteroids while on treatment. Patients applying a topical steroid to a large surface area or areas under occlusion should be evaluated periodically for evidence of HPA axis suppression.

Any of the side effects that are reported following systemic use of corticosteroids, including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children. Pediatric patients may be more susceptible to systemic toxicity from equivalent doses due to their larger skin surface to body mass ratios. As the safety and efficacy of Mefurosan in paediatric patients below 2 years of age have not been established, its use in this age group is not recommended. Local and systemic toxicity is common especially following long continued use on large areas of damaged skin, in flexures and with polythene occlusion. If used in childhood, or on the face, occlusion should not be used. If used on the face, courses should be limited to 5 days and occlusion should not be used. Long term continuous

therapy should be avoided in all patients irrespective of age. Topical steroids may be hazardous in psoriasis for a number of reasons including rebound relapses following development of tolerance, risk of centralised pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis careful patient supervision is important. As with all potent topical glucocorticoids, avoid sudden discontinuation of treatment. When long term topical treatment with potent glucocorticoids is stopped, a rebound phenomenon can develop which takes the form of a dermatitis with intense redness, stinging and burning. This can be prevented by slow reduction of the treatment, for instance continue treatment on an intermittent basis before discontinuing treatment. Glucocorticoids can change the appearance of some lesions and make it difficult to establish an adequate diagnosis and can also delay the healing. Mefurosan topical preparations are not for ophthalmic use, including the eyelids, because of the very rare risk of glaucoma simplex or subcapsular cataract.

INTERACTIONS WITH OTHER MEDICAMENTS
No known data.

PREGNANCY AND LACTATION
Since safe use of Mefurosan in pregnant women has not been established, topical corticosteroids should be used during pregnancy only if the potential benefit justifies potential risk to the fetus. Drugs of this class should not be used on pregnant patients in large amounts or for prolonged periods of time. It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breast milk. Systemically administered corticosteroids are secreted into breast milk in quantities not likely to have a deleterious effect on the infant. Nevertheless, a decision should be made whether to discontinue nursing or to discontinue Mefurosan, taking into account the importance of the drug to the mother.

ADVERSE REACTIONS
Local adverse reactions reported very rarely with Mefurosan cream include

paresthesia, pruritus and signs of skin atrophy. The following local adverse reactions have been reported infrequently with the use of other topical corticosteroids: Irritation, hypertrichosis, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infections, striae and milaria

SYMPTOMS AND TREATMENT OF OVERDOSE
Excessive, prolonged use of topical corticosteroids can suppress hypothalamic-pituitary-adrenal function resulting in secondary adrenal insufficiency which is usually reversible. If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application or to substitute a less potent steroid. The steroid content of each container is so low as to have little or no toxic effect in the unlikely event of accidental oral ingestion.

DOSAGE FORM AND PACKAGING
Cream.
Tube with net weight of 10 g.

STORAGE
Store below 30°C and in a dry place.

Manufactured by:
PT CAPRIFARMINDO LABS.
Jl. Industri Cimareme No. 8,
Desa Cipeundeuy, Kec. Padalarang,
Kabupaten Bandung Barat,
Indonesia

For:
PT SANBE FARMA.
Jl. Taman Sari No. 10, Bandung,
Indonesia

KEEP OUT OF REACH OF CHILDREN

JAUHI DARIPADA KANAK-KANAK

Date of Revision: April 2023

* BF 020 - EMY2

Country: Malaysia	Packaging: Insert	Dimension: 88 x 248 mm (double side printing)	Material: HVS paper 60 g	Color:  BLACK
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